

Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

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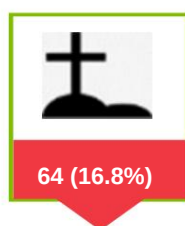
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°020/MVB_02/06/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (25)	• Ituri (17/36): Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara • North Kivu (7/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha • South Kivu (1/34): Miti-Murhesa
Reporting date	• June 3, 2026
Publication date	June 4, 2026



Total confirmed cases
for the 3 provinces



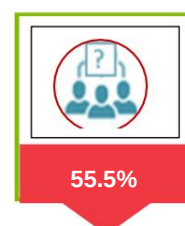
Cumulative deaths
among confirmed
cases and case fatality rate



Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Follow-up rate of
contacts for the 3
provinces

*Data currently being harmonized. **Location of these confirmed active cases is underway in order to harmonize statistics.

1. HIGHLIGHTS

- **Eighteen (18) new confirmed cases** were reported on June 3, 2026. These cases are distributed in the health zones of **Bunia (12), Rwampara (4), Damas (1) and Lita (1)**.
- **Zero deaths** among the 18 new confirmed cases. However, **two deaths** occurred among patients already confirmed (1 in Bunia, 1 in Rwampara), bringing the cumulative number of confirmed deaths to 64.
- **Official discharge of the first recovered case** of Ebola virus disease in Goma (North Kivu). The patient has been reintegrated into society in the community in the presence of health authorities and partners.
- **Security incidents:** (1) Attack on EDS teams in Katana (South Kivu) with the coffin abandoned and the body handled by the community. (2) Popular uprising in Mongbwalu (Ituri) with material damage to two vehicles (MSF and Red Cross). (3) Discovery of two lifeless bodies in Bunia (Salongo neighborhood) awaiting EDS.
- **In North Kivu**, response activities were paralyzed in Beni, Butembo and Oicha following opposition demonstrations and the massacre of approximately 24 civilians in Mbau (Oicha health zone), attributed to suspected ADF.
- **Deployment of a RADIONE machine (4 modules) and Glove box** in the health zone of Mongbwalu, within the framework of the decentralization of laboratory diagnosis.
- **Two hundred and thirty-two (232) new contacts** listed in 5 ZS: Rwampara (93), Bunia (55), Nyankunde (45), Komanda (28) and Aungba (11).
- **Start of the first training session for EDS teams (35 agents)** in the health zones of Bunia and Rwampara.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

The Ituri province, located in northeastern DRC, shares a long and porous border with Uganda and South Sudan. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, the presumed starting point of the epidemic, is characterized by the presence of armed groups, frequent population movements toward Uganda, and numerous artisanal mining sites attracting migrant workers.

The current epidemic, caused by the **Bundibugyo ebolavirus**, was officially declared on **May 15, 2026**, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly **15 million**, with massive internal displacement and cross-border flows to neighboring countries. Ituri remains the national epicenter with **94.2% of confirmed cases** (359/381). The case fatality rate there is **13.9%** (50/359). North Kivu has a concerning case fatality rate of 68.4%, attributed to delays in treatment and the escape of confirmed cases from healthcare facilities.

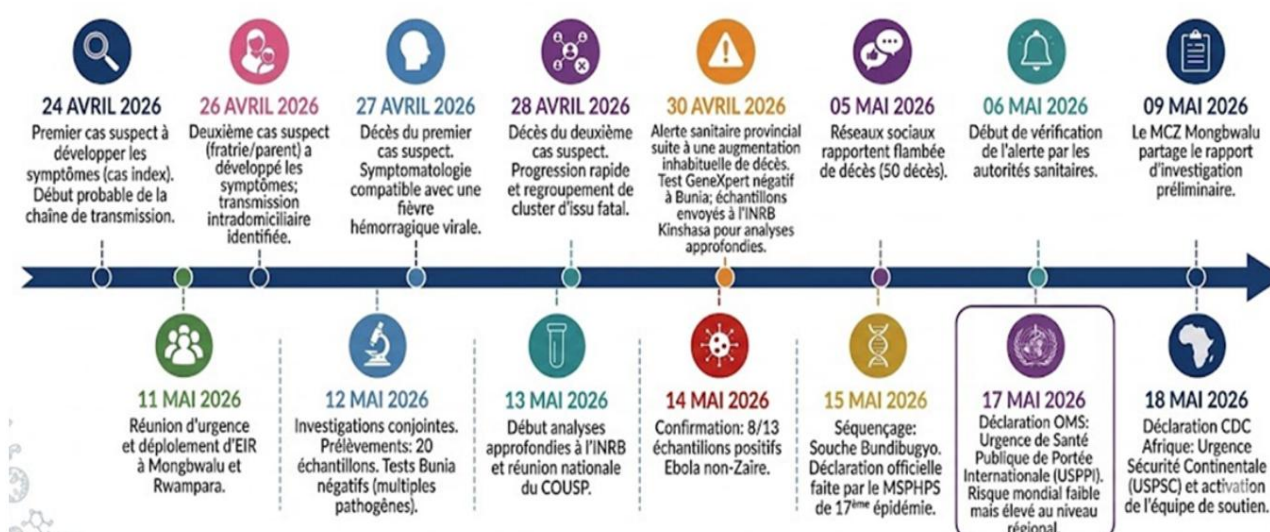


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 3, 2026

Province	Case confirmed	Deaths of confirmed	Lethality	affected zones	New cases (24h)
Ituri	359	50	13.9%	17/36 (47.2%)	18
North Kivu	19	13	68.4%	7/34 (20.6%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	381	64	16.8%	25/104 (24.0%)	18

3.2 Distribution by health zone (Cumulative as of 03/06/2026)

- **Ituri Province (359 cases):** Bunia (97), Rwampara (76), Mongbwalu (47), Nyankunde (22), Rimba (3), Damas (3), Lita (2), Bambu (2), Aru (2), Kilo (2), Mambasa (2), Nizi (2), Mangala (1), Aungba (1), Gety (1), Komanda (1), Logo (1). Ninety-four (94) additional cases are being ventilated.

- **North Kivu Province (19 cases):** Katwa (7), Beni (5), Butembo (2), Oicha (2), Kalunguta (1), Kyondo (1), Goma (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

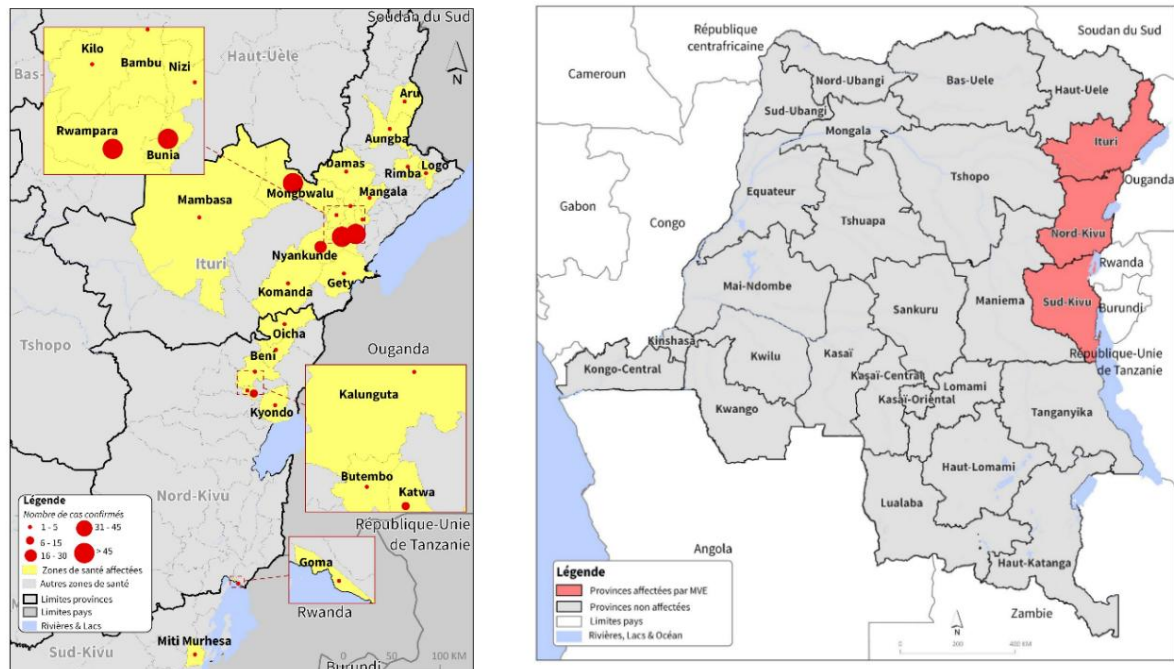
3.3 Demographic Analysis

The **18-49** age group is the most affected, with a **female predominance among the 18 new cases reported on June 3 (67% women)**. The overall trend remains a slight male predominance (~55% men) across all cumulative cases. Children under 5 years old represent 11% of the new cases reported that day (2 cases).

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, DRC, as of June 3, 2026

Province / Health Zone	Cumulative confirmed cases (n)	Deaths from cumulative confirmed cases (n)	Lethality (CFR, %)
ITURI			
1- Bunia	97	9	9.3
2- Rwampara	76	16	21.1
3- Mongbwalu	47	10	21.3
4- Nyankunde	22		4.5
5- Bamboo	2	1	100.0
6- Aru	2		50.0
7- Kilo	2	2	0.0
8- Nizi	2	1	0.0
9- Rimba	3	0	0.0
10- Mambasa	2		50.0
11- Mangala		0	0.0
12- Damas	1	0	0.0
13- Aungba		1	0.0
14- Gety		0	0.0
15- Komanda		0	0.0
16- Lita	3	0	0.0
17- Logo		0	0.0
Other ZS (non-ventilated)	1 1 1 2 1 94	0 0 0 10	10.6
Ituri subtotal	359	50	13.9
NORTH KIVU			
1- Katwa	7	5	71.4
2- Beni	5	3	60.0
3- Butembo	2	2	100.0
4- Oicha	2	2	100.0
5- Kalunguta			100.0
6- Kyondo		1	0.0
7- Goma		0	0.0
North Kivu subtotal	1 1 1 19	0 13	68.4
SOUTH KIVU			
1- Miti-Murhesa			33.3
South Kivu subtotal			33.3
TOTAL	33381	1 1 64	16.8

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of June 2, 2026

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

- **National / Ituri:** Strategic coordination meeting chaired by the Governor of Ituri. Debriefing to the Governor by the Director General of the INSP on the joint mission to Mongwalu. Continued work on developing the consolidated operational plan (CONOPS) and the prioritization exercise for health zones.
- **North Kivu:** Participation in the discharge ceremony for the recovered patient in Goma. Holding of the Incident Management System meeting at the DPS. Monitoring of the quarantine of travelers.
- **South Kivu:** Meeting with the Provincial Governor. Recommendation for direct funding of Activities. Meeting of the Health Cluster.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 3, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Alerts raised	166 *	237	6	409
Alerts investigated (24h)	88 *	216	6	310
Investigation rate (24h)	62% *	91%	100%	75.8%
Alerts confirmed	141	45	0	186

*Partial data for Ituri: 154 new alerts out of 166, investigation ongoing for alerts reported from the previous day.

Table 3. Contact tracing of confirmed cases as of June 3, 2026

Province	Contacts under follow-up	Contacts viewed	Follow-up rate (%)
Ituri	(n) 3	(24h)	53.0
North Kivu	837 533	2,032,343	64.4
South Kivu	181	150	82.9
Total	4,551	2,525	55.5

Challenges: Persistent low performance in Ituri (53.0%), although improving. In North Kivu (64.4%), significant disparities persist (Goma, Nyiragongo and Oicha at 100%, Kalunguta at 0%).

The escape of 2 confirmed cases and 17 suspects in Ituri, and of one confirmed case in South Kivu (CTE Lwiro), constitutes a major obstacle.

4.3 Laboratory

- **Ituri:** 82 samples collected and analyzed (18 positive, 61 negative, 3 invalid), representing a positivity rate of 22.0%. No samples are pending. Deployment of a RADIONE machine (4 modules) in Mongbwalu for decentralized testing.
- **North Kivu:** 39 samples collected in 24 hours. 10 tested in the Petit Nord (0 positive). **147 results pending in the Grand Nord**, average delay > 5 days. 100 swab kits received in Beni.
- **South Kivu:** Continued installation of the INRB mobile laboratory in Miti-Murhesa.

4.4 Infection Prevention and Control (IPC)

- **Ituri:** Briefing of 20 service providers on the wearing and removal of PPE (Nyankunde General Hospital). 4 EDS (Emergency Response System) interventions carried out out of 10 alerts in the Sayo Health District. Completion of work on 3 isolation units (Shari Health Center, Makabo Health Center, Lengabo Health Center in the Rwampara Health Zone). Training of 35 EDS agents.
- **North Kivu:** Average PCI score of **22.9** %. Briefing of 5 service providers from CM La Grâce (Far North) and 44 service providers in Karisimbi. PCI input shortage reported (critical situation).
- **South Kivu:** Scorecard evaluation of 19/35 establishments in 8 health zones. Briefing of 80 people (56 RECO, 24 U-REPORT) on standard precautions and the wearing/removal of PPE.

4.5 Holistic care

Table 4. Patient movement within healthcare facilities as of June 3, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	208	8	6	222
Total admissions (24h)	47	1	0	48
Deaths (24h):	11	0	0	11
Exits (24h): No cases	5	0	0	5
Outcomes (24h): Cured			0	1
Exits (24h): Escapees	0 19	10	1	20
Total outflows	35	1	1	37
Patients in isolation (End of Day),	220	8	5	233
including confirmed cases (NC	57	4	1	62
+ AC) and suspected cases	163	4	4	171

Key features of the holistic approach:

- **19 escapes** recorded in Bunia (2 confirmed, 17 suspected).
- **First recovered case discharged from Goma** on June 3, 2026.
- **South Kivu:** 1 confirmed case who escaped from the LWIRO Ebola Treatment Center. 2 babies receiving formula are being monitored in a daycare center. adapted. 17 high-risk contacts isolated at HGR FOMULAC KATANA.
- **Ituri:** Nutritional support (180 hot meals, 6 patients on a specific diet). Psychosocial support for 4 new confirmed cases and 11 suspected cases.

4.6 Risk Communication and Community Engagement (RCCE)

- **North Kivu:** 19,832 people reached through awareness campaigns (including 17,357 door-to-door visits to 7,352 households out of an expected 18,584, representing 40%). 1,648 students and 60 teachers reached. 241 Red Cross volunteers. **699 cases of resistance** recorded. 59 community alerts issued (4.2% response rate). 18 local radio stations involved. Home visits conducted in 448 households in Butembo (1,335 people reached).
- **Ituri:** Community dialogue with leaders of the Nyakasanza neighborhood (Bunia). Press conference by the Director General of the National Institute of Public Health (INSP) with 25 journalists. Extensive awareness campaign by the military governor against the rumor of the empty coffin.
- **South Kivu:** Community dialogue with 36 participants in Kahungu. Awareness-raising of 714 schoolchildren (339 girls) and 18 teachers.

Identified rumors: Belief in traditional herbal treatments; rumor of empty coffins buried by response teams (source: AS Muzimaria households, Bunia Health Zone); fear of neighbourhood chiefs calling response teams (Mongbwalu Health Zone); denial of the existence of Ebola virus disease.

4.7 Logistics

- **Ituri:** Official handover of 3 vehicles by the Governor to the Ituri CDPS; 2 allocated to the Rwampara and Nyankunde health zones. Delivery of IPC supplies to the Mambasa, Mandima, Niania, and Mongbwalu health zones. 25 vehicles and 4 ambulances made available. Fuel supply for field vehicles and the Bunia General Referral Hospital ambulance. Monitoring of the construction work at the Ebola Treatment Center at Bunia General Referral Hospital.
- **North Kivu:** Development of the distribution plan for inputs received in Beni.
- **South Kivu:** Plea for 2 additional vehicles and an Internet kit in Miti-Murhesa.

4.8 Security

- Unstable situation in the **Nizi and Bambu health zones**. The presence of armed groups in the territories Djugu, Irumu and Mambasa limit humanitarian access.
- **Major incident in Katana (South Kivu):** Attack on the Red Cross EDS team, withdrawal of coffin, manipulation of the body by the community.
- **Mongbwalu (Ituri):** Uprising of young people against the response team, windshields of two vehicles damaged by projectiles.
- **Bunia:** Attack at the cemetery which left **5 injured** among the response teams (reported on June 2).
- **North Kivu:** Massacre of approximately 24 civilians in Mbau (Oicha health zone) attributed to the ADF, paralyzing activities of retaliation.
- Security briefing for **19 experts** who arrived from Kinshasa.

4.9 PSEA (Prevention of Sexual Exploitation and Abuse)

- Risk analysis and PSEA briefing organized for internally displaced persons at ISP sites, Salama, Kigonze and Tsere.
- Briefing of 52 service providers at the Points of Entry (Ituri) and 41 members of the response team in Miti-Murhesa (South Kivu).

- PSEA briefing of 93 SGI MVE members (20 women), followed by the signing of the code of good practice conduct.
- Briefing of 33 community leaders (14 women) in Kavumu, and of 50 RECOs and leaders community (21 women) in Kashusha and Mbayo.
- Total: **863 people** briefed and signed the code of conduct (448 men, 375 women, 22 girls, 18 boys).

5. MAIN CHALLENGES

No.	Challenges	Impact	Action required
1	Reluctance to perform post-mortem sampling (swabs)	Delay in confirmation, underestimation of cases	Intensive community engagement, involvement of religious leaders
2	Insufficient capacity in Standardized CTEs	Potential saturation of existing CTEs	Acceleration of the construction of planned CTEs
3	Weakness in contact tracing (overall rate 55.5%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4	147 laboratory results pending in North Kivu (delay > 5 days)	Delay in confirmation and processing	Strengthening the supply chain, PCR activation RADIONE
5	Shortage of PCI supplies and sampling tubes at North Kivu	High risk of hospital-acquired infection	Urgent supplies
6	20 patient escapes (2 confirmed, 17 suspected in Ituri + 1 confirmed in South Kivu)	Community spread, distrust of the response	Securing CTEs, community engagement
7	Persistent insecurity (ADF, armed groups)	Uncovered areas, detection delays	enhanced security coordination
8	Supply shortage MEG in targeted HGRs	Weakening of overall care	A plea for MEG funding
9	Insufficient financial resources	Slowdown in operations	Urgent mobilization of resources
10	Incidents against EDS teams (Katana, Mongbwalu, Bunia)	Abandonment of bodies, risk of new chains of transmission	Community dialogue, team safety

6. PERSPECTIVES AND PRIORITY ACTIONS (Days 1-7)

Priority	Action	Responsible pillar	Due date
1	Training of service providers on the use of the RADIONE PCR	Laboratory	Day 2
2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Surveillance/CREC	Day 3
3	Briefing of community agents on Ebola surveillance and SBC (province Ituri)	Surveillance/CREC	Day 3
4	Supervised support from the CEO INSP in the technical pillars	Coordination	Continuous
5	Catch-up in contact tracing in Ituri (current rate 53.0%)	Monitoring	Day 7
6	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
7	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	Day 3
8	Catch-up plan for the 147 samples pending in North Kivu	Laboratory	Day 2
9	North Kivu: Rehabilitation continues CTE Katwa, Security Incident Response (ADF), PCI HGR training Virunga, CREC extension to 15,000 households	Multi-pillar	Day 3
10	South Kivu: Donation of 2 vehicles, installation of internet kits in Miti-Murhesa, communication credits	Logistics	Day 2
11	Ituri: Intensive community dialogue in areas of high resistance, securing of CTEs	CREC/Security	Day 2

Debriefing to the Governor of Ituri province by the Director General of INSP on the joint mission in the Mongbwalu health zone (INSP-SGI-WHO-UNICEF-WFP) focused on evaluating Ebola response activities.



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INFORMATION**

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